

EVERGREEN COMMUNITY CARE INC.

IT & CYBER SECURITY POLICY MANUAL

Acceptable Use · Password and Account Security · Device and Network Security · Privacy and Remote Access · Cloud Services · Data Backup and Disaster Recovery · Cybersecurity Incident Response · Electronic Records · Artificial Intelligence and Emerging Technology

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Review Date	July 1, 2027 (annual); also upon any funder directive, reportable cybersecurity incident or breach, or material change to agency IT systems
Applies To	All Evergreen Community Care residential care programs and 1–2 bed homes. All Youth Care Workers / Community Support Workers, Shift Leads, Program Coordinators, Program Managers, the Executive Director, board members, contractors, students, and volunteers who use agency devices, networks, accounts, or data.
Legislative Authority	Freedom of Information and Protection of Privacy Act (FOIPPA); Personal Information Protection Act (PIPA); Personal Information Protection and Electronic Documents Act (PIPEDA); E-Health (Personal Health Information Access and Protection of Privacy) Act; Criminal Code (unauthorized use of a computer, s.342.1); Canada's Anti-Spam Legislation (CASL); Copyright Act
Funder Standards	MCFD Standards for Staffed Children's Residential Services (SHSS) — records and privacy provisions; CLBC contract terms and standards — privacy, records, and information management provisions per the executed service contract
Related Documents	Health Care & Consent Policy Manual (ECC-HC-2026-001) — electronic health records; Medication Administration Manual (ECC-MED-2026-001) — Medication Administration Record systems; HR Policy Binder (ECC-HR-POL-2026-001) — onboarding and offboarding of system access; OHS Program Binder (ECC-OHS-POL-2026-002) — emergency response; Governance & Compliance Manual — records management and privacy compliance; Policy Interlinkage Map (Domain 8 — Information Technology)

 This manual governs how staff, contractors, students, and volunteers use Evergreen Community Care's technology, accounts, and data. Every person in our care has personal information — much of it health information — held on agency systems. A device left unlocked, a password shared, or a file sent to the wrong place is not a minor lapse; it is a privacy and safety incident. No agency device, account, or system is used in a way that is inconsistent with this manual. Any question this manual does not answer is

escalated to the Program Manager or the designated IT contact before proceeding.

TABLE OF CONTENTS

PART 1 — Policy Foundation	Sections 1–4: Policy Statement; Legislative and Regulatory Framework; Roles and Responsibilities; Key Definitions
PART 2 — Access and Acceptable Use	Sections 5–6: Acceptable Use of Technology (8.1); Password and Account Security (8.2)
PART 3 — Devices and Network	Sections 7–8: Device and Equipment Management (8.3); Network and Wi-Fi Security (8.4)
PART 4 — Privacy and Remote Access	Sections 9–10: Privacy Protection and Personal Information Security (8.5); Remote Access and Mobile Device Use (8.6)
PART 5 — Cloud, Backup and Continuity	Sections 11–13: Cloud Storage and Third-Party Services (8.7); Data Backup and Recovery (8.9); IT Disaster Recovery and Business Continuity (8.10)
PART 6 — Incident Response	Section 14: Cybersecurity Incident and Data Breach Response (8.8)
PART 7 — Records and Emerging Technology	Sections 15–16: Electronic Records Management (8.11); Artificial Intelligence and Emerging Digital Technologies (8.12)
PART 8 — Quality Assurance and Training	Sections 17–18: QA Indicators and Monitoring; Annual Review and Staff Training
PART 9 — Appendices	A — IT Asset Inventory and Assignment Log · B — Acceptable Use and Confidentiality Acknowledgement · C — Cybersecurity Incident / Breach Report Form · D — Password and MFA Setup Checklist · E — Cloud Service and Vendor Register · F — Annual IT Security Self-Audit Checklist · G — Policy Interlinkage Reference Table

PART 1 — POLICY FOUNDATION

1. POLICY STATEMENT AND GOVERNING PHILOSOPHY

Evergreen Community Care depends on technology to keep the people in our care safe: Medication Administration Records, health and consent records, incident reports, scheduling, and communication with families and funders all run on agency systems. That same technology, used carelessly or attacked successfully, can expose the personal and health information of children, youth, and adults who did not choose to share it. This manual sets the minimum standard for how every person who touches agency technology — staff, contractors, students, board members, and volunteers — protects it.

Principle	What It Means in Practice
Confidentiality by default	Information about a person in care is shared only on a need-to-know basis, through approved agency systems, never through personal accounts, personal devices, or unsecured channels.
Least privilege	Staff are given the minimum system access needed to do their job — no more. Access is granted at onboarding, adjusted on role change, and removed on the last working day, without exception.
Security is everyone's job	Locking a screen, reporting a phishing email, and not sharing a password are not IT's responsibility alone — they are every staff member's responsibility, every shift.
Assume breach, plan for it	The agency plans and trains as though a cybersecurity incident will happen, because eventually one will. Fast, correct reporting limits harm; a hidden or delayed incident does not.
Privacy and safety are the same goal	A privacy breach involving a child or youth in MCFD care, or an adult in CLBC care, is a safety incident. It is reported, documented, and reviewed with the same seriousness as any other critical incident.
Technology serves the person in care	New tools, apps, and AI systems are adopted only where they support safe, high-quality care and are approved through this manual's process — not because they are convenient for staff.

 No staff member, contractor, student, or volunteer uses a personal email account, personal messaging app, personal cloud storage, or personal device to store or transmit information about a person in care, except where this manual's Remote Access and Mobile Device provisions (Section 10) are followed exactly.

2. LEGISLATIVE AND REGULATORY FRAMEWORK

Authority	Relevance and Key Requirements
Freedom of Information and Protection of Privacy Act (FOIPPA)	Governs the collection, use, disclosure, storage, and security of personal information handled under public-body funding arrangements, including MCFD-funded services. Personal information must, subject to funder direction, be stored and accessed in a manner consistent with FOIPPA requirements, including data residency and access controls.

Authority	Relevance and Key Requirements
Personal Information Protection Act (PIPA)	BC's private-sector privacy law. Applies to Evergreen's collection, use, and disclosure of personal information about clients, staff, and volunteers not otherwise governed by FOIPPA. Requires reasonable security safeguards, consent for collection, and breach response.
Personal Information Protection and Electronic Documents Act (PIPEDA)	Federal private-sector privacy law; relevant where information crosses provincial or national borders, including with some cloud service providers.
E-Health (Personal Health Information Access and Protection of Privacy) Act	Additional protection for health information, cross-referenced with the Health Care & Consent Policy Manual (ECC-HC-2026-001). Electronic health information is held only in approved systems per Section 15.
Criminal Code (Canada), s.342.1	Unauthorized use of a computer system is a criminal offence. Sharing credentials to allow unauthorized access, or accessing a system or record outside the scope of one's role, may constitute a Criminal Code violation as well as a policy breach.
Canada's Anti-Spam Legislation (CASL)	Governs commercial electronic messages sent on the agency's behalf; relevant to any fundraising, recruitment, or community communication sent by email.
Copyright Act	Governs use of licensed software, images, and materials on agency systems; unlicensed software is never installed on agency devices.
MCFD SHSS / CLBC contract standards	Both funders require records and information about children, youth, and adults in care to be kept secure, confidential, and available for funder review; both require prompt notification of any privacy or security incident affecting a person in care.

3. ROLES AND RESPONSIBILITIES

Role	IT and Cyber Security Responsibilities
Youth Care Worker / Community Support Worker	Use only agency-approved devices, accounts, and systems for any information about a person in care; lock every screen when stepping away; use a unique, strong password and enable multi-factor authentication (MFA) where offered; never share login credentials; report any suspicious email, message, or system behaviour to the PC immediately; never install software on agency devices.
Shift Lead	All worker responsibilities, plus: confirm the medication and shift-log systems are used correctly and left secured at shift end; escalate any device fault, lost device, or suspected phishing attempt to the PC the same shift.
Program Coordinator (PC)	Maintain the IT Asset Inventory (Appendix A) for the home; ensure new staff complete IT onboarding (account setup, MFA, acceptable-use acknowledgement) before receiving system access; ensure departing staff access is disabled the same day per the HR offboarding process; investigate and report suspected incidents per Section 14; complete the Annual IT Security Self-Audit Checklist (Appendix F) for

Role	IT and Cyber Security Responsibilities
	the home.
Program Manager (PM)	Approve device and account assignments; approve any new software, app, or cloud service before it is used with information about a person in care (Section 11); lead the initial response to any cybersecurity incident or breach reported in their programs; ensure staff complete annual IT security training; notify the funder of any reportable privacy or security incident.
Designated IT Contact / IT Provider	Configures and maintains agency accounts, devices, network security controls, and backups; manages the password and MFA standard (Section 6); leads technical containment and recovery for cybersecurity incidents; maintains the Cloud Service and Vendor Register (Appendix E); advises the ED on new technology and AI requests (Section 16).
Executive Director (ED)	Approves this manual annually; approves any technology acquisition or contract with access to personal information; is the final decision-maker on breach notification to funders, regulators, and affected individuals; ensures the agency carries appropriate cyber liability coverage; reports material incidents to the Board.
Board of Directors	Receives an annual IT and cyber security summary from the ED; is briefed without delay on any material or reportable cybersecurity incident per the agency's governance policy.
All contractors, students, and volunteers	Complete the Acceptable Use and Confidentiality Acknowledgement (Appendix B) before receiving any system access; follow this manual to the same standard as staff for any device, account, or information they are given access to.

4. KEY DEFINITIONS

Term	Definition as Used in This Manual
Agency system	Any device, account, application, network, or cloud service owned, issued, licensed, or approved by Evergreen Community Care for agency use.
Personal information	Information about an identifiable individual, including a person in care, staff member, volunteer, or family member — names, contact details, health information, incident records, and images.
Multi-factor authentication (MFA)	A login method requiring two or more independent proofs of identity — typically a password plus a one-time code from an app or text message.
Cybersecurity incident	Any event that compromises, or could compromise, the confidentiality, integrity, or availability of an agency system or the information on it — including malware, phishing, unauthorized access, lost or stolen devices, and system outages with a security cause.

Term	Definition as Used in This Manual
Privacy breach	The unauthorized collection, use, disclosure, or disposal of personal information, whether by a cybersecurity incident, human error, or theft.
Least privilege	Granting a user only the system access required for their specific role, and no more.
Shadow IT	Any software, app, or cloud service adopted for agency work without going through the approval process in Section 11 or Section 16.
Endpoint	Any device that connects to the agency network or accesses agency accounts — laptops, tablets, phones, and workstations.

PART 2 — ACCESS AND ACCEPTABLE USE

5. ACCEPTABLE USE OF TECHNOLOGY (Interlinkage 8.1)

Corresponds to Domain 8 — Information Technology, Policy 8.1 in the ECC Policy Interlinkage Map. Links outbound to Human Resources 1.4 — staff onboarding triggers IT system access.

Use Standard	Requirement
Business use	Agency devices, accounts, and network access are provided for agency business — direct care, documentation, scheduling, and communication. Limited, incidental personal use is permitted only where it does not involve information about a person in care and does not interfere with work duties.
Prohibited use	Agency systems are never used to: access, store, or transmit information about a person in care outside approved systems; visit illegal, discriminatory, or explicit content; harass or discriminate against any person; conduct outside business; install unlicensed or unapproved software; or bypass security controls.
Email and messaging	Agency email is used for all work communication involving a person in care. Personal email and personal messaging apps (e.g., personal WhatsApp, SMS, social media messengers) are never used to send information about a person in care.
Social media	Staff never post images, names, or identifying details of a person in care on personal or agency social media without documented consent per the agency's communications policy. Staff do not identify their employer in a way that could connect a person in care to Evergreen without PM approval.
Monitoring	The agency may monitor use of agency systems for security and compliance purposes, consistent with BC privacy law and the employment agreement. Staff should have no expectation of privacy in agency systems used for agency purposes.
Consequences	Misuse of agency technology is addressed through the HR progressive discipline process; misuse that endangers a person in care, or that constitutes a criminal offence, is reported per Section 14 and may result in immediate suspension of access and termination.

6. PASSWORD AND ACCOUNT SECURITY (Interlinkage 8.2)

Corresponds to Domain 8, Policy 8.2. Links outbound to Human Resources 1.4 — password setup is completed as part of onboarding, before system access is granted.

Requirement	Standard
Unique credentials	Every user has their own individual login for every agency system. Shared logins are never used, except for a documented shared device account with no access to personal information (e.g., a shift-log kiosk), approved in advance by the PM.
Password strength	A minimum of 12 characters, combining upper and lower case letters,

Requirement	Standard
	numbers, and a symbol, or a passphrase of four or more unrelated words. Passwords are never reused across agency systems or shared with personal accounts.
Multi-factor authentication (MFA)	MFA is enabled on every account that offers it, without exception, for email, the Medication Administration Record system, the electronic health/consent record system, and any cloud storage or finance system.
No sharing	Passwords, MFA codes, and PINs are never shared with a colleague, family member, or anyone else, including to 'help out' a coworker who is locked out — a locked-out staff member is directed to the PC or IT contact for a reset.
Storage of credentials	Passwords are never written on paper kept near a device, saved in an unencrypted document, or stored in a browser on a shared device. An agency-approved password manager is used where provided.
Account lockout and reset	Accounts lock after 5 failed login attempts. Resets are performed only by the PC or IT contact after verifying the requester's identity — never based on an email or message request alone.
Offboarding	All account access for a departing staff member, contractor, student, or volunteer is disabled on their last working day, coordinated between the PM and the IT contact per the HR offboarding checklist. Shared or generic account passwords known to the departing person are changed the same day.

PART 3 — DEVICES AND NETWORK

7. DEVICE AND EQUIPMENT MANAGEMENT (Interlinkage 8.3)

Corresponds to Domain 8, Policy 8.3. Device assignment and inventory feed the IT Asset Inventory (Appendix A) and the agency's asset management process.

Requirement	Standard
Assignment and inventory	Every agency device (laptop, tablet, phone) is recorded on the IT Asset Inventory (Appendix A) with its assigned user, serial number, and program. The PC verifies the inventory at every Annual IT Security Self-Audit (Appendix F).
Screen lock	Every device automatically locks after no more than 5 minutes of inactivity and requires a password, PIN, or biometric to unlock. Staff manually lock a device whenever stepping away, without exception.
Encryption	Full-disk encryption is enabled on every agency laptop and desktop; device-level encryption is enabled on every agency tablet and phone.
Physical security	Devices are never left unattended in a vehicle, public space, or unlocked area. Devices not in active use are stored in a locked location within the home.
Loss or theft	A lost or stolen device is reported to the PC and IT contact immediately — the same shift — so remote lock or wipe can be initiated. This is treated as a cybersecurity incident per Section 14 regardless of whether information about a person in care is confirmed to be accessible.
Software installation	Only the IT contact installs or approves software on agency devices. Staff do not install apps, browser extensions, or software, including from personal accounts.
End of life	Devices being retired or reassigned are wiped by the IT contact using a secure-erase process before disposal, donation, or reassignment, and removed from the Asset Inventory with the disposal method recorded.
Personal devices (BYOD)	Personal devices are not used to access agency systems containing information about a person in care, except where the Remote Access provisions (Section 10) are followed, with PM approval and MFA in place.

8. NETWORK AND WI-FI SECURITY (Interlinkage 8.4)

Requirement	Standard
Separate networks	Each home operates a separate, password-protected staff Wi-Fi network for agency devices and business use, and a separate guest/youth network for personal devices used by people in care or visitors. The two networks never share a password.
Router security	Default router passwords and administrator credentials are changed

Requirement	Standard
	at setup; the router firmware is kept current by the IT contact; WPA3 (or WPA2 minimum) encryption is used.
Public Wi-Fi	Agency devices do not connect to open or unsecured public Wi-Fi networks to access information about a person in care. Where travel requires a connection, a secured personal hotspot or VPN is used.
Firewall	A firewall is active on every agency network and every agency device, maintained by the IT contact.
Network access review	The IT contact reviews devices connected to each home's staff network at least annually, and removes any unrecognized device.

PART 4 — PRIVACY AND REMOTE ACCESS

9. PRIVACY PROTECTION AND PERSONAL INFORMATION SECURITY (Interlinkage 8.5)

Corresponds to Domain 8, Policy 8.5. Links outbound to Governance & Compliance 9.5 — privacy protection feeds the agency's overall compliance framework.

Principle	What It Means in Practice
Need-to-know access	Staff access only the records of the people in care they directly support, unless broader access is required for their role (e.g., PM, PC) and has been granted per Section 6.
Minimum necessary sharing	Information about a person in care is shared with a third party (school, health provider, family) only to the extent necessary for that specific purpose, and only through an approved channel.
Secure transmission	Personal or health information sent by email is sent only to a verified agency, funder, or professional email address, using encryption or a secure portal where the receiving party offers one. Information about a person in care is never sent by unencrypted personal email or text.
Printed and physical information	Printed records containing personal information are not left unattended, are stored in locked cabinets when not in use, and are shredded (not placed in regular recycling or trash) when no longer needed, per the agency's records retention schedule.
Screens and visibility	Screens displaying information about a person in care are positioned away from public or shared view and are locked whenever the user steps away.
Third-party and vendor access	Any vendor or contractor with access to agency systems containing personal information has a signed confidentiality agreement and access limited to what their work requires, reviewed under Section 11.

10. REMOTE ACCESS AND MOBILE DEVICE USE (Interlinkage 8.6)

Requirement	Standard
Approved remote access only	Agency systems containing personal information are accessed remotely only through an agency-approved method (agency device, agency account with MFA, and — where required by the IT contact — a VPN).
Personal devices	A personal device may be used only for functions the PM has specifically approved (e.g., viewing a schedule), never for the Medication Administration Record, the health/consent record, or any document containing personal information about a person in care, unless a secure, MFA-protected agency app is used.
Working off-site	Staff working off-site (e.g., appointments, on-call) use agency-issued devices; documentation involving a person in care is completed on the agency device or on paper that is returned and secured the same shift — never drafted on a personal device or personal notes app.

Requirement	Standard
Auto-lock and remote wipe	All devices approved for remote access have auto-lock enabled and are enrolled in the agency's remote lock/wipe capability where technically available.
Text messaging	Agency business text messages about scheduling or logistics are permitted; personal information about a person in care is never sent by SMS/text, which is not a secure channel.

PART 5 — CLOUD, BACKUP AND CONTINUITY

11. CLOUD STORAGE AND THIRD-PARTY SERVICES (Interlinkage 8.7)

Corresponds to Domain 8, Policy 8.7. Links outbound to Finance & Procurement 5.9 — cloud storage costs and contracts are tracked through asset management.

Requirement	Standard
Approval required	No new cloud service, app, or software that will store or process information about a person in care, staff, or the agency is used until it is reviewed and approved by the PM and IT contact, and added to the Cloud Service and Vendor Register (Appendix E).
Data residency and privacy terms	Before approval, the IT contact confirms the provider's data storage location, security certifications, and data-processing terms are consistent with FOIPPA/PIPA requirements and funder expectations. Preference is given to Canadian-hosted services where personal information is involved.
Personal cloud accounts prohibited	Personal Google Drive, Dropbox, iCloud, or similar accounts are never used to store agency files or information about a person in care.
Vendor agreements	Any cloud or software vendor with access to personal information has a written agreement addressing confidentiality, data security, breach notification, and data return or destruction on contract end, held by the ED.
Periodic review	The IT contact reviews the Cloud Service and Vendor Register at least annually, removing services no longer in use and confirming access lists are current.

12. DATA BACKUP AND RECOVERY (Interlinkage 8.9)

Corresponds to Domain 8, Policy 8.9. Links outbound to Finance & Procurement 5.9 — data backup and cloud storage link to asset management.

Requirement	Standard
Backup schedule	All agency systems holding records about a person in care (Medication Administration Record, health/consent records, incident reports, shift logs) are backed up automatically at least daily by the IT contact or the relevant software vendor.
Backup location	Backups are stored in a location separate from the primary system (a separate cloud region or off-site location), so a single event cannot destroy both the live data and the backup.
Restore testing	The IT contact tests that a backup can actually be restored at least annually, and documents the test result.
Retention	Backups are retained per the agency's records retention schedule and applicable funder requirements — health and medication records are retained per ECC-MED §6/§7 (7 years; 10 years for controlled drug records).

Requirement	Standard
Local device backup	Staff save agency documents to the approved agency system (not the local device only), so information is captured by the backup process rather than existing only on one device.

13. IT DISASTER RECOVERY AND BUSINESS CONTINUITY (Interlinkage 8.10)

Corresponds to Domain 8, Policy 8.10. Links outbound to Health, Safety & Emergency 7.8 — IT disaster recovery links to OHS emergency response; inbound from 7.8 — disaster recovery links to IT disaster recovery.

Requirement	Standard
Recovery priorities	In a system outage or disaster, the priority order for restoration is: (1) any system needed for immediate health and safety of people in care, including the Medication Administration Record and emergency contact information; (2) communication systems; (3) all other administrative systems.
Paper fallback	Every home maintains a printed emergency information sheet (medication, allergies, consent contacts) that is updated whenever the electronic record changes, so care is never delayed by a system outage. See ECC-HC §21 and ECC-MED for emergency procedures.
Recovery roles	The IT contact leads technical recovery; the PM leads program-level continuity decisions (e.g., temporary paper documentation); the ED leads funder and stakeholder communication during an extended outage.
Recovery time targets	Systems supporting direct health and safety are targeted for restoration within 4 hours of a reported outage; other administrative systems within 2 business days. Targets are reviewed annually against actual performance.
Testing	The IT disaster recovery plan is reviewed and, where practicable, tested (e.g., a tabletop exercise) at least annually, coordinated with the agency's broader business continuity planning under Governance & Compliance 9.12.

PART 6 — INCIDENT RESPONSE

14. CYBERSECURITY INCIDENT AND DATA BREACH RESPONSE (Interlinkage 8.8)

Corresponds to Domain 8, Policy 8.8. Feeds Governance & Compliance 9.4 — enterprise risk management, and any incident involving a person in care also follows the agency's critical incident reporting process.

 Report first, investigate second. Any suspected cybersecurity incident — a suspicious email already clicked, a lost device, unexpected system behaviour, or a possible unauthorized disclosure — is reported to the PC or IT contact immediately. Staff never attempt to hide, quietly fix, or wait to see if an incident 'resolves itself.'

Step	Action	Timeline
1	Staff who suspect or discover an incident (phishing, malware, lost/stolen device, unauthorized access, accidental disclosure) immediately disconnect the device from the network if safe to do so, and report it to the PC or IT contact — do not attempt to fix it themselves.	Immediately
2	PC notifies the PM and the IT contact. The IT contact begins technical containment (disabling accounts, isolating the device, forcing password resets as needed).	Within 1 hour
3	PM and IT contact assess scope: what system, what information, whose information (staff, person in care, family), and whether personal or health information was or could have been accessed.	Same day
4	If the incident involves, or may involve, personal information about a person in care, the PM notifies the ED, who determines funder notification (MCFD social worker / CLBC) and whether the incident meets the threshold for notifying affected individuals or the Office of the Information and Privacy Commissioner, in consultation with legal counsel as needed.	Within 24 hours of confirmation
5	The incident is documented on the Cybersecurity Incident / Breach Report Form (Appendix C): what happened, systems and data affected, containment actions, and notifications made. Any reportable incident involving a person in care also follows the funder's critical incident report timeline.	Before shift end; funder report per contract timeline
6	PM and IT contact complete a post-incident review within 10 business days: root cause, what worked, what to change (technical controls, training, or this manual), and confirm any corrective action is complete.	Within 10 business days

PART 7 — RECORDS AND EMERGING TECHNOLOGY

15. ELECTRONIC RECORDS MANAGEMENT (Interlinkage 8.11)

Corresponds to Domain 8, Policy 8.11. Links outbound to Governance & Compliance 9.6 — electronic records link to records management and compliance; and to Quality Assurance 6.8 — electronic records feed QA data collection.

Requirement	Standard
Approved systems only	Records about a person in care (health, consent, medication, incident, service planning) are created and stored only in approved agency systems named in this manual and the related policy manuals — never in personal notes apps, unapproved spreadsheets, or ad hoc documents.
Version and access control	Electronic records use system-level access controls and audit logs where available, so who accessed or changed a record can be identified. Shared documents (e.g., shift logs) are edited only by authorized staff, with entries dated and initialed.
Retention and disposal	Electronic records are retained per the agency records retention schedule and funder requirements, and securely deleted (not simply moved to a device trash folder) when the retention period ends and disposal is authorized by the PM.
Data quality	Records are entered promptly, accurately, and factually, consistent with the documentation standards in ECC-HC and ECC-MED. Electronic records carry the same accuracy and completeness expectations as paper records.
Cross-reference, not duplication	Where information already exists in one system (e.g., a medication record in the MAR system), it is referenced rather than re-entered or duplicated in another, reducing the risk of conflicting or outdated copies.

16. ARTIFICIAL INTELLIGENCE AND EMERGING DIGITAL TECHNOLOGIES (Interlinkage 8.12)

Corresponds to Domain 8, Policy 8.12. Links outbound to Management & Leadership 10.6 — AI and digital technology adoption links to the agency's change management process.

Requirement	Standard
Approval required before use	No AI tool (chatbot, transcription, drafting, or analytics tool) is used with information about a person in care, staff, or agency operations until it is reviewed and approved by the ED, on the IT contact's advice, and added to the Cloud Service and Vendor Register (Appendix E).
No identifying information into unapproved tools	Staff never enter a person in care's name, identifying details, health information, or any personal information into a public or unapproved AI tool, including for drafting or summarizing notes.
Human review of AI output	Any AI-assisted output used in agency work (a draft summary, a template) is reviewed and verified by a staff member before being relied upon or entered into a record — AI output is a starting point, never a final record of fact.

Requirement	Standard
Vendor due diligence	Any approved AI or emerging technology vendor is assessed for data residency, retention of inputs for model training, and security certifications, using the same standard as Section 11 cloud service approvals.
Transparency	Where an AI tool is used in a way that affects a person in care or their record, this is disclosed to the PM and documented, consistent with the agency's commitment to transparent, accountable use of technology in care.

PART 8 — QUALITY ASSURANCE AND TRAINING

17. QA INDICATORS AND MONITORING

Indicator	Standard / Target
MFA coverage	100% of accounts with MFA available have MFA enabled, verified at each Annual IT Security Self-Audit.
Offboarding timeliness	100% of departing staff/contractor/volunteer access disabled on the last working day.
Incident reporting timeliness	100% of suspected cybersecurity incidents reported to the PC/IT contact the same shift they are discovered.
Backup restore testing	At least one successful restore test per year per critical system.
Staff training completion	100% of staff complete annual IT security and privacy training (Section 18) by their review due date.
Unapproved technology (shadow IT)	Zero unapproved cloud services or AI tools found in use at each Annual IT Security Self-Audit; any found are reviewed and either approved or discontinued within 10 business days.

These indicators are reviewed by the PM and IT contact quarterly, and roll up into the agency's overall Quality Assurance framework and Enterprise Risk Management review (Governance & Compliance 9.4).

18. ANNUAL REVIEW AND STAFF TRAINING

Requirement	Standard
Onboarding training	Every new staff member, contractor, or volunteer completes IT and cyber security orientation — covering acceptable use, password/MFA setup, and incident reporting — before receiving system access, and signs the Acceptable Use and Confidentiality Acknowledgement (Appendix B).
Annual refresher	All staff complete annual refresher training covering current phishing and social-engineering tactics, password/MFA practices, and this manual's key requirements.
Policy review	This manual is reviewed annually by the ED and IT contact, and immediately upon any reportable cybersecurity incident, funder directive, or material change to agency systems, per Management & Leadership 10.6 (change management).
Awareness communication	The IT contact circulates timely alerts to staff on active phishing campaigns or emerging threats relevant to the sector.

PART 9 — APPENDICES

Appendix A — IT Asset Inventory and Assignment Log: Device type, serial number, assigned user, program/home, date issued, encryption status, date retired/disposed.

Appendix B — Acceptable Use and Confidentiality Acknowledgement: Signed by every staff member, contractor, student, and volunteer before system access is granted, confirming they have read and will comply with this manual.

Appendix C — Cybersecurity Incident / Breach Report Form: Date/time discovered and reported, description, systems and data affected, containment actions, notifications made (funder, ED, affected individuals, OIPC), root cause and corrective action.

Appendix D — Password and MFA Setup Checklist: Step-by-step checklist used at onboarding to confirm unique password and MFA are configured on every required system.

Appendix E — Cloud Service and Vendor Register: Every approved cloud service, software, or AI tool with access to agency or personal information — vendor, purpose, data residency, contract/renewal date, confidentiality agreement on file (Y/N).

Appendix F — Annual IT Security Self-Audit Checklist: Completed annually by the PC for each home — asset inventory verified, MFA coverage, screen-lock settings, network/Wi-Fi separation, backup restore test, unapproved software check.

Appendix G — Policy Interlinkage Reference Table

Cross-references between this manual's sections and the ECC Policy Interlinkage Map (Domain 8 — Information Technology), for use alongside the Policy Interlinkage Map Final document.

This Manual	Interlinkage ID	Connects To
§5 Acceptable Use of Technology	8.1	→ Human Resources 1.4 (staff onboarding triggers IT system access)
§6 Password and Account Security	8.2	→ Human Resources 1.4 (onboarding)
§7 Device and Equipment Management	8.3	→ Finance & Procurement 5.9 (asset management)
§8 Network and Wi-Fi Security	8.4	Supports all domains via secure connectivity
§9 Privacy Protection and Personal Information Security	8.5	→ Governance & Compliance 9.5 (privacy compliance)
§10 Remote Access and Mobile Device Use	8.6	Supports Client Care & Clinical and Program Operations field work
§11 Cloud Storage and Third-Party Services	8.7	→ Finance & Procurement 5.9 (asset management)
§14 Cybersecurity Incident and Data Breach Response	8.8	→ Governance & Compliance 9.4 (enterprise risk management)
§12 Data Backup and Recovery	8.9	→ Finance & Procurement 5.9 (asset management)
§13 IT Disaster Recovery and Business Continuity	8.10	→ Health, Safety & Emergency 7.8 (OHS emergency response)

This Manual	Interlinkage ID	Connects To
§15 Electronic Records Management	8.11	→ Governance & Compliance 9.6 (records/compliance); → Quality Assurance 6.8 (QA data collection)
§16 Artificial Intelligence and Emerging Digital Technologies	8.12	→ Management & Leadership 10.6 (change management)

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